

NEURO FORM 1

1. A

diffuse muscle atrophy and fasciculation (lower motor lesion)
brisk deep tendon reflex (upper motor)
no sensory abnormalities

= AMYOTROPHIC LATERAL SCLEROSIS

2. I

polymyositis

I thought of Lambert-Eaton also
but the rapid presentation in 6 weeks plus no mention about any chest issue so...polymyositis

3. E

chronic subdural hematoma

can not appear in first day cause bridging rupture vein is low pressure system
accumulate over time then compress ... neurological symptoms appear then
evacuation of hematoma and amazing outcome of condition

4. B

Guillain-Barré

- sensory involvement exclude polymyositis and amyotrophic and polio
- spinal stenosis (neurogenic claudication)

5. E

Normal aging
easy ques

6. E

the lady has a relapsing attack of M.S = I.V steroid

7. C

jaundice ... confused lethargic.. asterix

8. C

Herpes simplex encephalitis ... (fever , temporal lobe involvement on EEG , white blood cell elevation mainly lymphocyte in
csf analysis ,) Herpes associated with temporal lobe haze ..

9. D

(Restless leg syndrome

treatment options for Restless leg syndrome

Dopamine agonists

(Pramipexole , Ropinirole , Rotigotine transdermal patch)

Alpha-2-delta calcium channel ligands

(Gabapentin , Pregabalin)

10. A

shingles .. oral acyclovir

11. A

Arsenic Poisoning:

The primary target organs for arsenic toxicity are the **gastrointestinal tract, heart, skin, bone marrow, kidneys, and peripheral nervous system.**

Acute toxicity typically starts in the gastrointestinal system and includes nausea, vomiting, abdominal pain, and diarrhea. These symptoms are soon followed by dehydration, hypotension, and **QTc prolongation**.

In chronic poisoning, the peripheral neurologic complaints and skin manifestations are usually more prominent than the gastrointestinal symptoms.

Chronic exposure to arsenical compounds has occurred at work, usually through inhalation of arsenic-containing vapors or dusts, or through non-work environmental exposures, such as drinking arsenic-contaminated water.

Skin lesions

Hyperpigmentation or hypopigmentation can be an early manifestation (picture 1). Hyperkeratoses and scaling, particularly diffusely on the palms and soles, also are quite characteristic

Skin carcinomas and Bowen's disease (squamous cell carcinoma in situ) (picture 1) are associated with latent effects of arsenic poisoning

Also, nail **changes (Mee's or Beau's Lines)**

Neurologic manifestations — A symmetrical sensorimotor polyneuropathy is one of the most prominent symptoms of arsenic poisoning and can develop one to three weeks after acute poisoning or insidiously from chronic exposures.

Sensory symptoms tend to present first and to predominate.

In more severe forms, the pain is more intense, particularly with even light touch, so that affected persons are unable to walk because of intense burning pain in the soles.

Cancer (Skin - UB - Lung - Liver)

Cardiovascular — Some patients with acute promyelocytic leukemia treated with arsenic trioxide have been found to develop QTc prolongation and ventricular arrhythmias

12. C

high protein in csf

slow conduction velocity (demyelination)

this case (Guillain-Barré variant) not typical one

I think this one

acute inflammatory demyelinating polyneuropathy . The typical clinical features are a progressive, fairly symmetric muscle weakness accompanied by absent or depressed deep tendon reflexes.

The **cardinal clinical features of Guillain-Barré syndrome (GBS)** are progressive, fairly symmetric muscle weakness accompanied by absent or depressed deep tendon reflexes. Patients usually present a few days to a week after onset of symptoms. The weakness can vary from mild difficulty with walking to nearly complete paralysis of all extremity, facial, respiratory, and bulbar muscles.

13. C

pregnancy and failure to lactate ... pituitary problem

Sudden hemorrhage into the pituitary gland is called pituitary apoplexy. Hemorrhage often occurs into a pituitary adenoma. In its most dramatic presentation, apoplexy causes the sudden onset of excruciating headache, diplopia due to pressure on the oculomotor nerves, and hypopituitarism.

14. E

probably severe muscle sprain

4-6 weeks with good analgesia

NO red flag no sciatica

this question explanation well covered in UWORLD

15. C

huntington ... caudate nucleus .. GABA

16. E not sure

unconscious , vomiting , sever HTN
always think of intracranial bleeding

u can solve the question by answering intubation, he is not conscious lowering intracranial pressure accomplished acutely with hyperventilation pco2 of 30 : 35 will cause v.c of cerebral vessels .. decrease ICP
it should be used in moderation and for limited time
Kaplan internal medicine book page 342

more info :

methods to decrease ICP

elevate patient head

i.v mannitol

hyperventilation will reduce paCO2 so vasoconstriction and decrease cerebral blood flow

Mannitol is the most commonly used hyperosmolar agent for the treatment of intracranial hypertension.

More recently, hypertonic saline also has been used in this circumstance

Intravenous bolus administration of mannitol lowers the ICP in 1 to 5 minutes with a peak effect at 20 to 60 minutes. The effect of mannitol on ICP lasts 1.5 to 6 hours,

when urgent reduction of ICP is needed, an initial dose of 1 g/kg body weight should be given. Arterial hypotension (systolic blood pressure < 90 mm Hg) should be avoided.

Intravenous **mannitol** is the treatment of choice to lower increased intracranial pressure, quickly and effectively lowering ICP

Hyperventilation

Hyperventilation to a PaCO2 of 25 to 30 mmHg causes dramatic and **rapid lowering of ICP. However, the effect only lasts for minutes to a few hours.**

Hyperventilation decreases PaCO2, which can induce constriction of cerebral arteries by alkalinizing the CSF. The resulting reduction in cerebral blood volume decreases ICP. Hyperventilation has limited use in the management of intracranial hypertension, however, because this effect on ICP is time limited, and because **hyperventilation may produce a sufficient decrease in CBF to induce ischemia.**

dexa injection to decrease ICP better used in increase ICP related to tumor or infection

17. H

atrophic gastritis .. vit b 12 deficiency

18. C

Neisseria meningitidis .. Antimicrobial chemoprophylaxis of close contacts of sporadic cases of meningococcal disease is the primary means for prevention of meningococcal disease

Close contacts include a) household members, b) anyone directly exposed to the patient's oral secretions

Antimicrobials commonly used for chemoprophylaxis are rifampin, ciprofloxacin, ceftriaxone

Ciprofloxacin 500 mg in a single dose is probably the easiest option in adults. Children could receive either a single IM injection of ceftriaxone or 4 oral doses of rifampin over 2 days, according to body weight.

Antimicrobials commonly used for chemoprophylaxis are rifampin, ciprofloxacin, ceftriaxone, minocycline, and spiramycin.

When oral rifampin (4 doses in 2 d) was compared with a single IM dose of ceftriaxone for prophylaxis, follow-up cultures indicated that ceftriaxone was significantly more effective. Ceftriaxone may provide an effective alternative to rifampin for prophylaxis in people in close contact with patients with meningococcal meningitis.^[23]

<http://emedicine.medscape.com/article/1165557-treatment#d11>

19. A

botulism

Botulinum neurotoxins, causative agents of botulism in humans, are produced by *Clostridium botulinum*, an anaerobic spore-former Gram positive bacillus. Botulinum neurotoxin poses a major bioweapon threat because of its extreme potency and lethality; its ease of production, transport, and misuse; and the need for prolonged intensive care among affected persons. A single gram of crystalline toxin, evenly dispersed and inhaled, can kill more than one million people. The basis of the phenomenal potency of botulinum toxin is enzymatic; the toxin is a zinc proteinase that cleaves neuronal vesicle associated proteins responsible for acetylcholine release into the neuromuscular junction. As a military or terrorist weapon, botulinum toxin could be disseminated via **aerosol** or by contamination of water or food supplies, causing

widespread casualties.

<http://www.ncbi.nlm.nih.gov/pmc/articles/PMC3028942/>

botulism :

The muscle weakness of botulism characteristically starts in the muscles supplied by the **cranial nerves**. A group of twelve nerves controls **eye movements**, the **facial muscles** and the muscles controlling **chewing** and **swallowing**. **Double vision**, **drooping of both eyelids**, loss of facial expression and swallowing problems may therefore occur. In addition to affecting the **voluntary muscles**, it can also cause **disruptions in the autonomic nervous system**. This is experienced as **a dry mouth and throat** (due to decreased production of saliva), **postural hypotension** (decreased blood pressure on standing, with resultant lightheadedness and risk of blackouts), and eventually **constipation** (due to decreased **peristalsis**).^[3] Some of the toxins (B and E) also precipitate **nausea**, **vomiting**,^[3] and **difficulty with talking**. The weakness then spreads to the arms (starting in the shoulders and proceeding to the forearms) and legs (again from the thighs down to the feet).^[3]

Severe botulism leads to reduced movement of the **muscles of respiration**, and hence problems with **gas exchange**. This may be experienced as **dyspnea** (difficulty breathing), but when severe can lead to **respiratory failure**, due to the buildup of unexhaled **carbon dioxide** and its resultant depressant effect on the brain. This may lead to **coma** and eventually death if untreated.^[3]

Clinicians frequently think of the symptoms of botulism in terms of a classic triad: **bulbar palsy** and descending paralysis, lack of fever, and clear senses and mental status ("clear sensorium").^[4]

soman nerve gas (cholinergic activity like organophosphorous)

Immediate signs and symptoms of soman exposure

Although soman can have a camphor or fruity odor, the odor may not be noticeable enough to give people sufficient warning of a toxic exposure.

People exposed to a low or moderate dose of soman by breathing it in (inhalation), swallowing (ingestion), or skin absorption may experience some or all of the following symptoms within seconds to hours of exposure:

Abnormally low or high blood pressure

Blurred vision

Chest tightness

Confusion

Cough

Diarrhea

Drooling and excessive sweating

Drowsiness

Eye pain

Headache

Increased urination

Nausea, vomiting, and/or abdominal pain

Rapid breathing

Runny nose

Slow or fast heart rate

Small, pinpoint pupils

Watery eyes

Weakness

Even a tiny drop of nerve agent on the skin can cause sweating and muscle twitching where the agent touched the skin.

Exposure to a large dose of soman by any route may result in these additional adverse health effects:

Convulsions

Loss of consciousness

Paralysis

Respiratory failure possibly leading to death

Showing these signs and symptoms does not necessarily mean that a person has been exposed to soman.

<http://www.bt.cdc.gov/agent/soman/basics/facts.asp>

20. B

-bright illumination will increase agitation

-**haloperidol**: Antipsychotic agents have limited efficacy and are associated with increased mortality in patients with dementia. However, alternatives are limited when symptoms are severe, disabling, and/or threatening patient or caregiver safety despite safer interventions. When antipsychotic drugs are deemed necessary, we suggest low doses of olanzapine or risperidone after informing families of the mortality risk (Grade 2B). Short term use when possible, with regular reassessments of risks and benefits, is advised.

- use of restraints as needed : Physical restraints are rarely indicated in the care of patients with dementia and should be used only for patients who pose an imminent risk of physical harm to themselves or others.

Physical restraints should be used only as a last resort, if at all, as they frequently increase agitation and create additional problems, such as loss of mobility, pressure ulcers, aspiration, and prolonged delirium. In one study, restraint-use among patients in a medical inpatient unit was associated with a three-fold increased odds of persistent delirium at time of hospital discharge **Alternatives to restraint use, such as constant observation (preferably by someone familiar to the patient such as a family member), may be more effective. Not nurses.**

uptodate

21. A

cavernous sinus thrombosis

upper resp infection Sinusitis Spread to cavernous sinus

vision symptoms cranial n 3 and 6 , fever , Bad HEADACHE

22. E

Carpal tunnel syndrome

management :

wrist splint and NSAID first line

not effective inject steroid

still bad .. nerve conduction studies and surgery

For patients with CTS not treated with surgery who have an **inadequate response to wrist splinting**, we suggest a **single injection with methylprednisolone (40 mg) as the next therapeutic option rather than oral glucocorticoids (Grade 2B)**. For patients who decline injection therapy, we suggest treatment with oral glucocorticoids (**uptodate**)

23. C

multiple sclerosis ... MRI

24. A

family history low back pain

limited spinal cord flexion

tenderness at tendon insertion point (enthesitis)

esr .. 55

- growing pain : Growing pains are defined as pain awakening a child at night (or from naps) who is otherwise without any manifestation of musculoskeletal problems

-juvenile arthritis : symptoms for 3 weeks make this answer wrong

25. B not sure

- choice a (liver HCC to bone not common)

-choice c (myeloma ... lytic lesion not blastic)

- choice d (x ray for lung cancer ... the man was healthy 2 months ago and **well differentiated** adenocarcinoma and lung !)

prostate cancer or colorectal cancer

prostate cancer very common metastasis to spine

he is old .. and may be the lesion not on rectal part to feel it by digital ex

at least I do this rx and if -ve ----- investigate the colon (look for blood in stool or colonoscopy)

26. A

endocarditis **Embolic stroke**

treatment : antibiotic for endocarditis

27. F

HIV + multi focal brain lesion (toxoplasmosis)

28. F

organophosphorous poisoning

29. C

the man lift his son and something happened in his cervical spines

compressive cervical radiculopathy is recognized to be a common source of arm pain with or without sensory and motor dysfunction.

A radiculopathy is a pathologic process affecting the nerve root. Most radiculopathies arise from nerve root compression (figure 2). The two predominant mechanisms of compressive cervical radiculopathy are cervical spondylosis and disc herniation

Cervical radiculopathy is a common cause of both acute and chronic neck pain. Lower cervical roots, particularly C7, are more frequently affected by compression than higher cervical roots

MRI is currently the study of choice in most patients for the initial neuroimaging evaluation of the cervical spine.

Clinical signs and symptoms in typical solitary cervical root lesions :

c7 :

-**pain** .. Neck, shoulder, **middle finger**, hand

-**numbness** .. Index and **middle finger**, palm

-**weakness** .. Elbow and wrist extension (radial), forearm pronation, wrist flexion

-**reflex affected** .. **Triceps**

30. B

neuroleptic malignant syndrome

31. A

We might need to do B if the patient ended up needing it and had the desire to do the treatment

Some terminally ill patients elect to have only palliative support, nothing invasive

Tracheostomy with indications in Duchenne including:

- Patient and clinician preference; the guidelines recommend the long-term use of noninvasive ventilation up to and including 24 hours daily in eligible patients
- Inability of patient to use noninvasive ventilation successfully
- Inability of the local medical infrastructure to support noninvasive ventilation
- Three failures to achieve extubation during critical illness despite optimum use of noninvasive ventilation and mechanically assisted cough
 - The failure of noninvasive methods of cough assistance to prevent aspiration of secretions into the lung and drops in oxygen saturation below 95 percent or baseline, necessitating frequent direct tracheal suctioning via tracheostomy

32. B

dysthymic disorder

minor depressive symptoms more than 2 years .. and she is functioning and work

33. B

gait problem , dementia , urinary

normal pressure hydrocephalus

34. B

the rinne and weber tests done .. sensori neural hear loss

the big clue is the schwannoma removed 5 years ago from the left neurofibromatosis type 2 cause bilateral shwanomma

35. E

Acute angle closure glaucoma

tonometry TO MEASURE IOP

gold standard diagnosis is gonioscopy (u see the narrow angle)

Place the patient supine

provide immediate treatment to reduce IOP

N.B

- Diagnosis of angle-closure glaucoma is established by gonioscopy.

Patients with signs and symptoms suggesting an acute attack of angle-closure glaucoma require emergency treatment by an ophthalmologist. A table to assist with the emergent management of severe acute angle-closure glaucoma is provided (table 2). We recommend emergency use of topical ophthalmic medications to reduce intraocular pressure (Grade 1C). These drugs may include a beta-blocker, an alpha agonist, and an agent to produce miosis. We also suggest systemic medication to decrease intraocular pressure, which may include oral or IV acetazolamide, IV mannitol, oral glycerol, or isosorbide (Grade 2C). Once the acute attack is controlled, definitive treatment for angle-closure glaucoma is a laser peripheral iridotomy to provide a small drainage hole through the iris.

36. D

Acute Gastroenteritis ... dehydration (hypovolemia bp 60 /40 , sunken ant fontanel and eye , dry mucous membrane) ... abnormality in Na & k ... seizure

Dehydration can lead to serious complications

Seizures : Electrolytes — such as potassium and sodium — help carry electrical signals from cell to cell. If your electrolytes are out of balance, the normal electrical messages can become mixed up, which can lead to involuntary muscle contractions and sometimes to a loss of consciousness.

37. D

Cervical spine involvement is relatively common in RA, especially in longstanding disease, compared with the very infrequent involvement that occurs in the thoracolumbar spine or sacroiliac joints.

Symptoms of pain and stiffness in the neck are the most typical manifestation, but disease affecting the joints of the cervical spine can be of critical clinical importance, as longstanding disease may lead to instability and cause symptoms related to subluxation such as neck pain, stiffness, and radicular pain. If the subluxation is causing spinal cord compression, there may be signs of long tract involvement such as hyperreflexia or up going toes on Babinski testing.

38. F

simple partial seizure

Post-stroke seizures typically follow a localisation related (focal) seizure

absence seizure ... staring eyes (generalized)

atonic .. sudden drop (generalized)

complex partial .. post ictal state not present in the

question myoclonus .. sudden jerky movement

psychogenic ...

39. B

the huge difference in blood pressure in both arms

Subclavian steal is frequently asymptomatic and may be discovered incidentally on ultrasound or angiographic examination for other indications, or it may be prompted by a clinical examination finding of reduced unilateral upper limb pulse or blood pressure. **In some cases**, patients may develop upper limb ischemic symptoms due to reduced arterial flow in the setting of subclavian artery occlusion, or they may develop neurologic symptoms due to posterior circulation ischemia associated with exercise of the ipsilateral arm

Vasovagal syncope (—classical) refers to syncope triggered by emotional or orthostatic stress such as venipuncture (experienced or witnessed), painful or noxious stimuli, fear of bodily injury, prolonged standing, heat exposure, or exertion.

intermittent vasovagal reactions ::

This is in contrast to the —atypical form of reflex syncope (also referred to as —non-classical presentation).

Some patients, especially those who are older, have recurrent episodes of syncope without an identifiable cause or trigger. (uptodate)

Cardiovascular syncope :

Presence of definite structural heart disease

Sudden onset palpitation immediately followed **by** syncope

40. E

typical vasovagal attack ... (phlebotomy induce it)

41. A

acute ischemic neurologic symptoms

- choices b & c are wrong (intravenous thrombolytic therapy intravenous thrombolysis with **alteplase** for eligible patients ([table 1](#)) with acute ischemic stroke, including those with isolated extracranial or intracranial cervical artery dissection, provided that treatment is initiated within 3 hours of clearly defined symptom onset (**Grade 1A**). For patients who cannot be treated in less than 3 hours, we suggest intravenous alteplase therapy, provided that treatment is initiated within 3 to 4.5 hours of clearly defined symptom onset)

HOWEVER

For patients who cannot be treated with intravenous thrombolysis within 4.5 hours and have angiographically demonstrated extracranial or intracranial dissection and associated **ischemic stroke symptoms but no signs of major early infarction or subarachnoid hemorrhage on a baseline CT or MRI scan, we suggest intra-arterial **alteplase** therapy**, provided that treatment can be started **within 6 hours**

- choice e (warfarin as immediate therapy it take 3 day to anticoagulate)

More info :

For patients with acute ischemic neurologic symptoms caused by **extracranial** dissection, we suggest early treatment with antithrombotic therapy using aspirin rather than anticoagulation. However, some experts, including one of the authors of this topic (DL), prefer anticoagulation rather than aspirin in this setting. The choice between antiplatelet and anticoagulant therapy should be guided by the clinical experience of the treating physician and by patient values and preferences, comorbid conditions, and tolerance of these agents.

- Antithrombotic treatment (whether aspirin or anticoagulation) must be delayed until 24 hours after thrombolytic therapy but can be started immediately for those who are not treated with thrombolytic therapy.

- Most reports of antiplatelet therapy for acute cervical artery dissection have employed daily aspirin at various doses; there are few data regarding other antiplatelet agents such as clopidogrel, dipyridamole, or combinations of these agents. For patients treated with aspirin in the acute phase, we suggest continuing long-term antiplatelet therapy using aspirin, clopidogrel, aspirin-extended-release dipyridamole, or cilostazol for secondary prevention of stroke.

- unfractionated Acute anticoagulation may be achieved with either subcutaneous low molecular weight heparin such as enoxaparin (1 mg/kg twice daily) or dalteparin (100 U/kg twice daily) or with intravenous heparin (dose-adjusted to achieve a goal activated partial thromboplastin time of 1.5 to 2 times control). Transition to warfarin (dose adjusted for a goal International Normalized Ratio [INR] of 2.5 with an acceptable range of 2 to 3) can be pursued in the subacute period for clinically stable patients. For patients treated with anticoagulation in the acute phase, we suggest stopping warfarin and starting long-term antiplatelet therapy after six months of anticoagulation, as long as symptoms are not recurrent and the arterial lesion is thrombosed or healed. For patients with persistent vascular luminal stenosis, irregularity, or dissecting aneurysm, we suggest continuing anticoagulation with repeat imaging at regular intervals. We use transcranial Doppler, duplex Doppler, CT angiography, and MR angiography to help us decide the status of the arterial system prior to discontinuing anticoagulation therapy

uptodate

It is not good choice to choose dipyridamole alone

medscape :

Once contraindications to anticoagulation have been ruled out, the accepted management of proven or suspected spontaneous VAD consists of anticoagulant therapy in those patients who are not also affected by the complication of subarachnoid hemorrhage.

This approach is intended to prevent thrombogenic or embolic occlusion of the vertebrobasilar network and subsequent infarction of posterior CNS structures, brain stem, and cerebellum.

42. A

11 y girl . bitemporal visual defect .. calcified lesion

43. G

Transient global amnesia (TGA) is a benign, self-limited selective disturbance of anterograde amnesia that occurs in middle-aged and elderly subjects.

- Older age is the only consistently identified risk factor.
- The primary site of neurologic functional disturbance is the medial temporal lobe. The pathogenesis of this transient disruption is unknown. Current theories include arterial ischemia, venous congestion, and migraine, but no theory explains all of the clinical features.
- The event is characterized by a profound disruption in **anterograde memory**; patients are typically disoriented to place and will repeatedly ask where they are and why they are there. Retrograde amnesia is variably present. **Other cognitive and executive functions are intact**. Patients do not lose self-awareness. Resolution typically occurs after several hours.
- Patients with TGA should be observed until the episode resolves. Investigations should include laboratory tests to exclude intoxication, infection, and metabolic derangements. Brain MRI with DWI is recommended to exclude stroke or other structural causes.
- The prognosis after TGA is felt to be benign, and the risk of recurrence is low. No treatment or driving restriction is required

44. A

Options for the secondary prevention of TIA or ischemic stroke caused by large artery disease include revascularization (mainly for symptomatic internal carotid artery stenosis) and intensive medical therapy (ie, antiplatelet, antihypertensive, and statin therapy)

Selected patients with recently symptomatic carotid stenosis of 50 to 99 percent who have a life expectancy of at least five years are generally treated with carotid endarterectomy. For patients with TIA or ischemic stroke having carotid endarterectomy, we recommend aspirin at a dose of 81 to 325 mg/day started before surgery.

For patients with TIA or ischemic stroke of atherothrombotic, lacunar (small vessel occlusive), or cryptogenic type, we recommend treatment with an antiplatelet agent. This issue and the choice among specific antiplatelet agents (ie, aspirin, the combination aspirin-extended-release dipyridamole, or clopidogrel)

For most patients with atrial fibrillation and a recent ischemic stroke or TIA, we recommend oral anticoagulation. We recommend aspirin for patients with atrial fibrillation and cardioembolic stroke who have contraindications to anticoagulant therapy

the pt carotid stenosis is 40 % so u go for aspirin

45. B

diabetic .. gastroparesis .. metoclopramide side effects (extrapyramidal side effects .. pseudoparkinsonism)

46. C Narcolepsy ..

I solve this ques wrong

acute weakness of neck and knee muscles make the confusion

my information about cataplexy is the sudden loss of postural tone and fall to ground with a trigger without loss of conscious

Cataplexy — Cataplexy is the most specific symptom of narcolepsy. Cataplexy is seen in approximately 80 percent of children with narcolepsy [2]. It typically emerges at the same time as excessive sleepiness or shortly thereafter.

Cataplexy is characterized by sudden, transient loss of muscle tone; the weakness or paralysis usually arises in response to strong emotions such as laughter, surprise, anger, fright, or anticipation of reward. Consciousness is fully preserved. Episodes usually last

a few seconds to three to five minutes. **The severity of attacks ranges from a slight head or shoulder drop to sudden collapse to the floor.**

Buccofacial involvement is prominent in children, manifested by the jaw dropping open, eyelid drooping (ptosis), head rolling, or tongue thrusting movements [23]. This has given rise to the term "cataplectic facies," which is a unique clinical feature of childhood presentations [23,24]. The usual triggering emotions are not always apparent in children, and the symptom complex is sometimes confused with tics or even myasthenia gravis. Over time, a more classic form of cataplexy often emerges

other sleep disorders :

Delayed sleep-wake phase disorder

Teenager with inability to fall asleep prior to midnight to 2 a.m., relatively well organized and normal sleep quality, inability to awaken fully prior to mid-morning; male predominance

Kleine-Levin syndrome

Teenager with periods of hypersomnia lasting 10 to 14 days and occurring two to three times per year; normal sleep-wake function in intervening periods; male predominance

During episodes of hypersomnia, patients commonly have cognitive disturbances, derealization, and severe apathy. They may also exhibit disinhibition in the form of hyperphagia and hypersexuality. The episodic nature of EDS in Kleine-Levin syndrome and the prominent cognitive and behavioral disturbances distinguish it from narcolepsy

47. B

CO poisoning: the question in winter time, so probably she is heating. CT shows bilateral globus pallidus hypo-density. <https://radiopaedia.org/articles/carbon-monoxide-poisoning-1>

48. A

to look up .. so u assess inferior oblique m.s
if it is 3 rd n prb there should be some ptosis
so entrapment of the inf oblique m.s the right one

N.B

Vertical diplopia in primary gaze suggests underactivity of the right or left inferior rectus, superior rectus, inferior oblique, or superior oblique [3]. If vertical separation is worse on right gaze, the right superior or inferior rectus or the left inferior or superior oblique may be underactive; separation that is worse looking to the right and down suggests underactivity of the right inferior rectus or left superior oblique.

If u looked closely u will find the right eye can look upward and to the right but the left eye can't; an action that is done solely by left IO, so A is the right answer.

49. D

diazepam

Benzodiazepines — Benzodiazepines are the most extensively studied drugs for the treatment of alcohol withdrawal. These drugs treat the psychomotor agitation associated with withdrawal and prevent progression from minor to major withdrawal

MINOR WITHDRAWAL SYMPTOMS — Minor withdrawal symptoms are due to central nervous system hyperactivity, and can include:

- Insomnia
- Tremulousness
- Mild anxiety
- Gastrointestinal upset; anorexia
- Headache
- Diaphoresis
- Palpitations

50. E

the acute onset of all half of face (nasolabial fold .. unable to close eye) ... bell's palsy
steroids and no investigation